DENVER PHYSICAL THERAPY & REHABILITATION

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PHYSICAL THERAPY EVALUATION

SUBMITTED FOR PRIOR AUTHORIZATION REQUEST

Patient: Christopher James Brown
DOB: June 18, 1990 (Age: 33)
MRN: MRN778899
Date of Evaluation: January 22, 2024
Insurance: Aetna Better Health (ID: AET456789012D)
Referring Physician: Dr. Rebecca Lynn Taylor, DPT
NPI: 7788990011

CHIEF COMPLAINT

Low back pain and right knee pain for 3 months, limiting work and daily activities.

HISTORY OF PRESENT ILLNESS

Mr. Brown developed low back pain 3 months ago (October 15, 2023) after lifting heavy boxes at work. Pain is located in the lumbar region, radiates to right leg, and is worse with sitting and bending. Right knee pain developed 1 month later (November 20, 2023), possibly due to compensatory gait changes from altered movement patterns secondary to low back pain.

PAST MEDICAL HISTORY

No significant past medical history reported. No previous episodes of low back pain or knee injuries.

CONSERVATIVE TREATMENT ATTEMPTED

1. **Rest and activity modification** - 6 weeks (October-November 2023)
2. **NSAIDs (ibuprofen 600mg TID)** - 8 weeks (October-December 2023)
3. **Muscle relaxants (cyclobenzaprine 10mg BID)** - 4 weeks (November-December 2023)
4. **Home exercise program** - 4 weeks (December 2023-January 2024) - limited compliance due to pain and fear of movement
5. **Heat and ice therapy** - ongoing since October 2023

6. Ergonomic assessment at work - completed December 2023, workstation modifications implemented

FUNCTIONAL ASSESSMENT SCORES

Assessment Tool	Score	Interpretation
Oswestry Disability Index (ODI)	42%	Moderate disability
Numeric Pain Rating Scale - Low Back	7/10	Severe pain
Numeric Pain Rating Scale - Right Knee	5/10	Moderate pain
Fear Avoidance Beliefs Questionnaire	28/66	High fear avoidance behaviors

PHYSICAL EXAMINATION

Posture: Anterior head posture, increased lumbar lordosis, guarded movement patterns

Lumbar Spine:
• Range of Motion: Limited flexion (60 degrees, normal 80-90), painful extension
• Palpation: Tenderness over L4-L5 paraspinal muscles
• Special Tests: Positive straight leg raise at 45 degrees on right

Right Knee:
• Inspection: Mild swelling, no obvious deformity
• Range of Motion: Limited flexion (110 degrees, normal 135)
• Special Tests: Positive patellar compression test
• Stability: Ligamentous tests negative

Strength Testing (Manual Muscle Testing):
• Hip abductors: 4/5 bilaterally
• Gluteus maximus: 4/5 bilaterally
• Quadriceps: 4/5 right, 5/5 left
• Core stability: Decreased endurance

Balance and Coordination:
• Single leg stance: < 10 seconds bilaterally (normal >30 seconds)
• Tandem walking: Unsteady with compensation

DIAGNOSTIC IMAGING

Lumbar X-ray (January 15, 2024):
• Mild disc space narrowing L4-L5
• No fracture or spondylolisthesis
• Normal vertebral alignment

Right Knee X-ray (January 18, 2024):
• Normal bone structures
• Mild joint space narrowing
• No fracture or dislocation

CLINICAL ASSESSMENT AND DIAGNOSIS

Primary Diagnosis:
• M54.5 - Low back pain (mechanical, work-related injury)

Secondary Diagnosis:
• M25.561 - Pain in right knee (compensatory due to altered gait patterns)

Clinical Impression:
Mr. Brown presents with mechanical low back pain with secondary knee pain due to compensatory movement patterns. He demonstrates significant functional limitations and has not responded adequately to conservative treatment over the past 3 months. The patient shows clear indications for skilled physical therapy intervention.

INDICATIONS FOR PHYSICAL THERAPY

1. **Moderate functional disability** (ODI 42%) impacting work and daily activities
2. **Failed conservative treatment** for 3 months with multiple interventions
3. **Significant strength and mobility deficits** requiring skilled intervention
4. **High fear avoidance behaviors** (FABQ 28/66) requiring specialized education
5. **Work-related functional limitations** affecting ability to perform job duties
6. **Potential for functional improvement** with specialized physical therapy care
7. **Secondary complications** (knee pain) requiring comprehensive treatment approach

SERVICES REQUESTED FOR PRIOR AUTHORIZATION

Prior Authorization Request ID: PA-AET-PT-2024-008
Payor: Aetna Better Health
Provider: Denver Physical Therapy & Rehabilitation (NPI: 3579024681)

CPT Code	Service Description	Quantity	Unit Price	Total Cost	Service Period
97110	Therapeutic procedure, 1 or more areas, each 15 minutes; therapeutic exercises to develop strength and endurance, range of motion and flexibility	24 sessions	$85.00	$2,040.00	01/29/2024 - 03/29/2024

97140	Manual therapy techniques (eg, mobilization/manipulation, manual lymphatic drainage, manual traction), 1 or more regions, each 15 minutes	12 sessions	$95.00	$1,140.00	01/29/2024 - 03/29/2024
TOTAL REQUESTED AUTHORIZATION				**$3,180.00**	

Place of Service: Office (Code 11)
Treatment Location: Denver Physical Therapy & Rehabilitation
Frequency: 3-4 sessions per week for 8 weeks

PROPOSED TREATMENT PLAN

Treatment Interventions:
1. **Manual therapy** (CPT 97140) for joint mobility and soft tissue restrictions
2. **Therapeutic exercise** (CPT 97110) for strength, stability, and endurance
3. **Postural re-education** and movement pattern training
4. **Ergonomic training** and work conditioning
5. **Pain education** and coping strategies
6. **Gradual return** to functional activities and work duties

Frequency and Duration:
• 3-4 sessions per week for 8 weeks
• Total of 24 therapeutic exercise sessions (CPT 97110)
• Total of 12 manual therapy sessions (CPT 97140)
• Treatment period: January 29, 2024 - March 29, 2024

Expected Outcomes:
• 50-75% improvement in Oswestry Disability Index
• Return to full work duties without restrictions
• Improved pain levels (target: <3/10)
• Restoration of normal movement patterns
• Prevention of chronic pain syndrome

MEDICAL NECESSITY JUSTIFICATION

Physical therapy is medically necessary for Mr. Brown due to his significant functional limitations, failed conservative treatment, and potential for improvement with skilled intervention. The patient's work-related injury has resulted in compensatory movement patterns affecting multiple body regions. Without appropriate treatment, he is at risk for developing chronic pain syndrome and permanent functional limitations. The proposed treatment plan addresses both primary and secondary diagnoses with evidence-based interventions.

PATIENT EDUCATION AND CONSENT

Patient has been counseled on the treatment plan, expected outcomes, and goals. He understands the importance of compliance with the therapy program and agrees to proceed with the recommended treatment. Patient verbalized understanding of home exercise program requirements and activity modifications.

Evaluating Therapist:

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Dr. Rebecca Lynn Taylor, DPT
Doctor of Physical Therapy
License #: PT12345CO
NPI: 7788990011
Date: January 22, 2024

Supervising Physician (if required):

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Print Name: _____
Date: _____